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Name: رشا محمد عبده محمد National ID: 26809281802561	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM

29/12/2025 G

Name	رنا عيسى	MRN	907/CO
Age	57 years	ID	010 11995413
Menopausal status	Post Menopausal	Phone	010 20641044
NRS SCORE		Marital status	Married
BMI		Family history	+ve
CO morbidity	DM -HTN.	REASON OF MDT	Post-operative

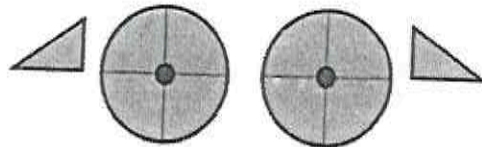
Clinical Data

BX: FOC & DCIS

R

L

ER: 0/3 PR: 4/8 HER 2: +ve Ki67:



TNM

Sonomammography: Post NACT.

Mets Workup: Post Rt MRM
→ Nipple total: free
Deep Margin: free

Recommendations: 3/12 = Extra-nodal Extension.
Partial response to therapy.
ypTis & ypN1a.

29/12/25

MDT OUTCOME & PLAN

non-PCR after TCM. &
① Port to R axilla & PLN
② Adjuvant Kadcyla L 12 cycles

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
 29/12/2025					

Name: رشا محمد عبده محمد		
MR. : 888		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته تاريخ وساعة الدخول : تاريخ وساعة الخروج :		
وبعد الكشف والفحوصات والمتابعة وجد انه postm rt breast cancer , ct3 n3 mo , er0/8 pr4/8 her2 3+ , rec NACT : TCHP on 2/12/2025 , RT MRM 121/2025 : yptis ypn1 recieved c1 kadcyla 216 mg and (IHC) on specimen of mrm , port pending FOR C2 KADCYLA TODAY LABS PENDING HX OF RT URETHRAL STONE FOR UROLOGY CONSULTATION today for C3 kadcyla , patient didnt recieve IHC (if for hormonal or not) and also PORT >> P C4 after labs + ECHO >> pharmacy C5 pharmacy after labs >> pharamacy Pharmacy Visit For C4 Kadcyla 216 mg Labs 4/5 ... accepted ECHO NEXT CYCLE postm rt breast cancer ct3 n3 vmo idc 2 er0/8 pr4/8 her23+ rec tchp on 2/12/2026 rt mrm yptis ypn1 3/12 er0/8 pr0/8 on mrm specimen er 59% 14/7 for c6 kadcyla		
تحريرا في ,		
	التاريخ : 19-09-2026	الوقت :
علياء احمد أبو النصر شهاب الدين		الطبيب المعالج :
يعتمد ,		

Patient Name	: الأستاذة / رشا محمد عبده محمد الشريف	Request No.	: 13002828
Patient No.	: 26809281802561	Request Date	: 26/03/2025
Path No.	: SR-498/25	Verification Date	: 08/04/2025
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 56 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass & right axillary lymph node.

Nature of Specimen: Tru-cut biopsy from breast mass & FNAC from right axillary L.N, Block & slide revision.

Gross Examination:

Three paraffin blocks labeled by (184)(200626-25) and six slides were received.

Microscopic Examination:

Right breast mass tru-cut biopsy:

Histological type: Invasive duct carcinoma.

Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.

Ductal carcinoma in situ: No.

Angioinvasion: Positive.

Perineural invasion: No.

Microcalcifications: Undetected.

IPT:

ER: No nuclear reaction in tumor cells (score 0/8)

PR: Weak nuclear reaction in 20% tumor cells (score 4/8)

Her2: Strong complete membrane reaction in >10 of tumor cells (score 3+)

Right axillary lymph node FNAC biopsy:

Cell block positive for few malignant epithelial cells.

Diagnosis:

Right breast mass & right axillary lymph node, Block & slide revision:

Right breast mass tru-cut biopsy:

Invasive duct carcinoma. Grade II, with positive lymphovascular invasion.

Verified by :

Dr.Toka Medhat



Patient Name : الأستاذة / رشا محمد عبده محمد الشريف	Request No. : 13002828
Patient No. : 26809281802561	Request Date : 26/03/2025
Path No. : SR-498/25	Verification Date : 08/04/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 56 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

IPT:

- ER: Negative(score 0/8).
- PR: Positive (score 4/8).
- Her2: Positive (score 3+).

-Right axillary lymph node FNAC biopsy:

Cell block positive for few malignant epithelial cells.

Signature

Dr. Rana Fathy.

Dr. Samia Ali.

Verified by :

Dr.Toka Medhat

Printing Date : 08/04/2025 13:21:07

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / رشا محمد محمد الشريف	Request No. : 13003751
Patient No. : 26809281802561	Request Date : 02/12/2025
Path No. : 3733/25	Verification Date : 09/12/2025
Sex : Female	Financial Category : نفقة الدولة - جوستاف روسي
Age : 57 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Post-NACT.
Nature of specimen: Right modified radical mastectomy.

Gross Examination:

- * Right modified radical mastectomy specimen measured 21x16x4 cm, with covering skin ellipse measured 18x10 cm. Nipple & areola looked unremarkable grossly.
- * Senalling showed a tumor bed site measured 8x7x2 cm, located 2 cm away from the deep margin.
- * Axilla: Axillary fat measured 8x5x2 cm, entangling 12 lymph nodes, the largest measured 1.5x1x0.5 cm.
- * 14 Blocks:
- 1:8- T.B.S., 9- N&A., 10- Deep margin., 11:14- ALNs.
- Gross was done by: Dr. Heba El-Shafeey.

Microscopic examination:

- * Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- * Ductal carcinoma in situ: 20% Residual high & intermediate grades ductal carcinoma in situ (Cribriform & clinging patterns with comedo type necrosis).
- Microcalcification: Present.
- Nipple and areola: Free.
- Deep margin: Free.
- Axillary lymph nodes: Positive viable metastatic deposits in three out of twelve lymph nodes (3/12) (The largest metastatic focus measuring 0.5 cm, with extra-nodal extension).
- Histologic response to therapy: Pathologic partial response to therapy.

Diagnosis:

Right breast cancer (Post-NACT). Right modified radical mastectomy:
Residual 20% high & intermediate grades ductal carcinoma in situ (Cribriform & clinging

Verified by :
Dr.Duaa Mohamed

Printing Date : 14/12/2025 13:15:50

Page 1 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name :	الأستاذة / رشا محمد عبد محمد الشريف	Request No. :	13003751
Patient No. :	26809281802561	Request Date :	02/12/2025
Path No. :	3733/25	Verification Date :	09/12/2025
Sex :	Female	Financial Category :	جوانف روسي - نفقة الدولة
Age :	57 Y	Department :	Out Patient
Referring Doc :		Registered by :	Mrs.Mayada Hegazy

Pathology Report

atenna with comedo type necrosis).
No residual viable remnants of the previously diagnosed invasive tumor.
Nipple and areola: Free.
Deep margin: Free.
Axillary lymph nodes: Positive viable metastatic deposits in three out of twelve lymph nodes (The largest metastatic focus measuring 0.5 cm, with extra-nodal extension).
Stage: ypTis, ypN1a.
RCB: 1.516, RCB class-II.

Signature

r. Walaa Ibrahim Ghoneim.
r. Heba El-Shafeey

Verified by :
Dr.Duaa Mohamed

g Date : 14/12/2025 13:15:50

Page 2 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Minr El Qadema - Cairn, Egypt

Name: Mrs. Rasha Mohamed Abdo

Date: 24/11/2025

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Right breast cancer, post NACT assessment study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- UOQ , posterior third speculated irregular high density mass lesion with surrounding distortion.
- Relatively thickened skin and coarsen trabeculae.
- **Ultrasound** demonstrated 10 -11 o'clock, ill defined heterogenous hypoechoic solid mass lesion with clips seen within its lateral border ,moderate internal vascularity on color Doppler study measuring 35 x 15 mm associated multiple enlarged axillary lymph nodes are noted , the most suspicious of which is seen involving level I measuring 10 x 6 mm with lost fatty hilum .
- Normal nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast with mottled appearance with 10 OC position well defined hypoechoic solid lesion showing internal macrocalcifications measuring 8 x 6 mm.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 6): Right breast suspicious looking lesion with pathological axillary lymphadenopathy.

Left breast (BIRADS 2): Benign looking lesion , possibly calcified fibroadenoma .

Best Regards,

DR. Israa Reda , M.Sc.



Patient Name: Rasha Mohamed Abdou Mohamed Al Sherif	Date : 08/04/2025
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 56 Year(s)
Medical ID: 956/2025	Pat. ID: 98761

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of right breast cancer.

I Chest:

- Right breast UOQ irregular mass lesion measuring 40 X 32 mm with enlarged right axillary level I, II and III lymph nodes the largest measures 1.6 X 1.1 cm.
- Few bilateral 2-3 mm pulmonary nodules.
- Prominent subcentimetric mediastinal lymph nodes.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Average sized liver showing homogenous parenchymal attenuation. No sizable focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys, suprarenal glands, aorta and IVC.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- Bulky uterus.
- Bilateral calcific adnexal region lesions.
- Bilateral hemipelvic sclerotic lesions the largest is at right iliac wing measuring 1.5 cm.

Opinion:

Reported case of right breast cancer revealed:

- Bilateral hemipelvic sclerotic lesions for bone scan.

?, ??? Patient ID: 10110225008395 DOB: 28-Sep-1968 Date: 12-May-2025
ent Lastname: محمد
ent Firstname: رشا
Patient ID: 10110225008395
Patient ID: 369425
e of Birth: 28/09/1968

cedure: 70030, Bone Scan
edical Question:
inical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (12/5/2025)
Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP
revealed:

No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
Degenerative changes are noted over the dorso-lumbar vertebrae.
Arthritic changes are noted over both shoulder and knee joints.
Both kidneys are visualized.

Conclusion:
No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated
Omnia Talaat, MD
Areej Mohamed

===== REPORT TEXT ENDE =====

اسم المريض ثلاثي: عبدالله عيسى
رقم الملف:

تقرير طبي

السيد/

تحية طيبة وبعد،،

إيماء للطلب المقدم من

بقسم رقم طبي

نتشرف بأن نوضح فيما يلي تقريراً عن حالته:

تاريخ وساعة الدخول

تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:

A 58 y old ♀, post menopausal, a car-
diac bread-cancer cT3N3 ER 0/8 PR 4/8
HER 2 +3. Received neo adjuvant TCHP
Rt MRM 12/2025 gTIS N1. MD detection
for PART 8 for Adjuvant Kadcyla for 12 cycles
last received Q8 on 14/9/2026 To receive
Kadcyla 4x weekly

التاريخ: 27/1/7

الوقت:

تحريراً في

الطبيب المعالج

يتمتع

مراجع
Dr. Kadi



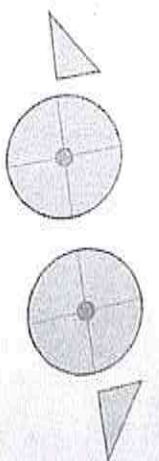
مملكة العربية السعودية
لجنة صحة المرأة المصرية
MDT FORM

البيانات الشخصية
المرأة
المرأة

MDT FORM 2019/2025 6

Name	ياسمين	MRN	90215054-18
Age	57 years	ID	010119954-18
Menopausal status	Postmenopausal	Phone	010208410344
MRS SCORE		Marital status	Married
BMI		Family history	None
CO morbidity	DM -H-T-N	REASON OF MDT	Postmenopausal CA 1A. CO 71

Clinical Data



TNM

Bx:	TDC DCIS		
ER:	PR:	HER 2:	Ki67:
0/8	4/8	+ve	

Sonomammography:
Post NACT.

Mets Workup: Post RT MRM
→ N. P. G. Gated: Free

Recommendations:
3/12 re Extra-nodal Extension
Partial response to therapy
48 Tis 24 PMTA.

MDT OUTCOME & PLAN
Non-BC after TCM. d.
① Post RT per Cal, axilla & PMN
② Axilla Radiotherapy in cycles

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatry
[Signature]		[Signature]			

20/12/2018